CareJiyo

Administrator manual

Configuration, access and controlled maintenance

ENGLISH

For authorised clinic administrators. Keep this volume separate from the everyday user manuals.

Two ways to use this manual

Business processes and task instructions: from page 2

Complete screen-by-screen guide: from page 32

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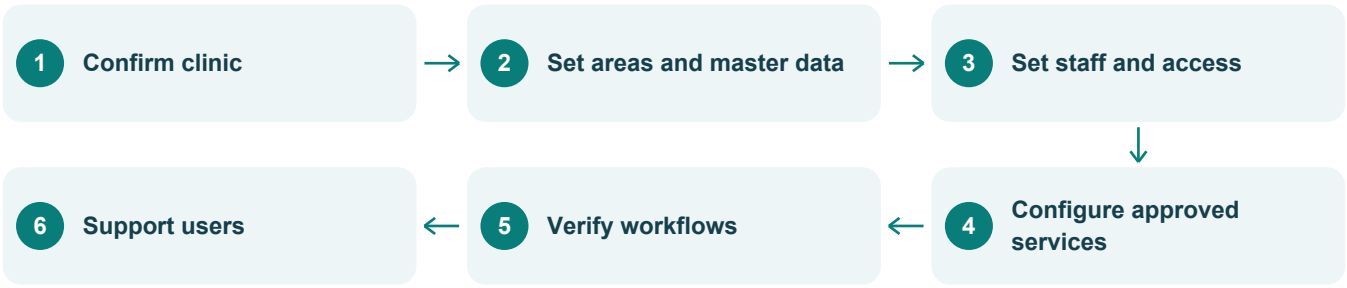
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CHAPTER 01

Administrator responsibilities and setup order

The normal process



In this chapter

Who should use this manual

Company, facilities, areas and initial checks

Read the process from step 1 onward. Your role, the record status and the enabled features determine which actions are available. Check the saved result before handing work to the next person.

1.1 Who should use this manual

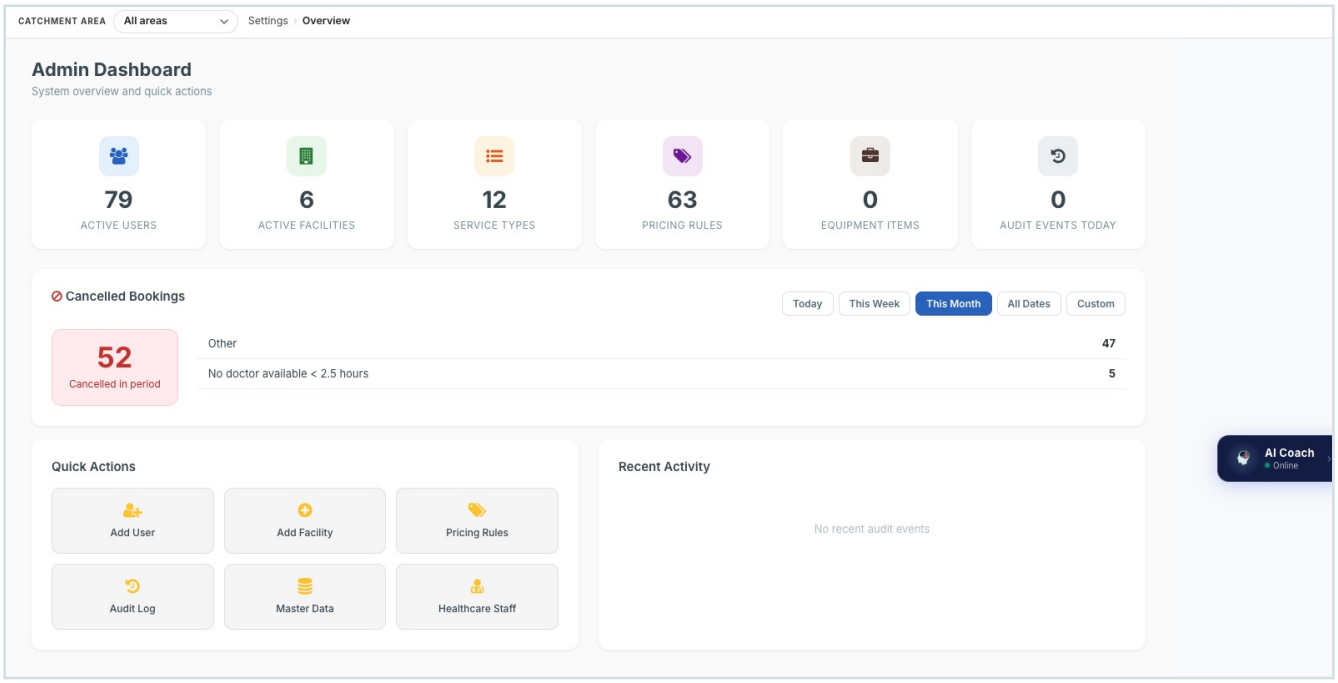


Figure 1. Administrator overview: common setup areas and recent activity.

This manual is for the clinic owner, authorised administrators and designated specialists who maintain configuration. It is separate from the everyday user manual because configuration can change what many people see and do. Keep distribution limited to the appropriate staff.

An administrator maintains the system; a clinical lead approves care content; a finance lead approves tariffs and financial procedures. Technical access does not replace those responsibilities. Ask the relevant owner to approve the business rule before changing it.

Use Settings > Overview to reach common setup areas. People controls accounts and staff; Master data controls approved reference values; Connections controls external services; Records provides history and lifecycle tools. Your company, Clinic setup and Activity history support the clinic workspace.

Before a change, note the affected company, role, catchment and existing value. Make a focused change, save it, then inspect the actual screen as the affected role. A menu being visible does not prove the user can read or change its records. Keep a brief record of the reason and result.

1.2 Company, facilities, areas and initial checks

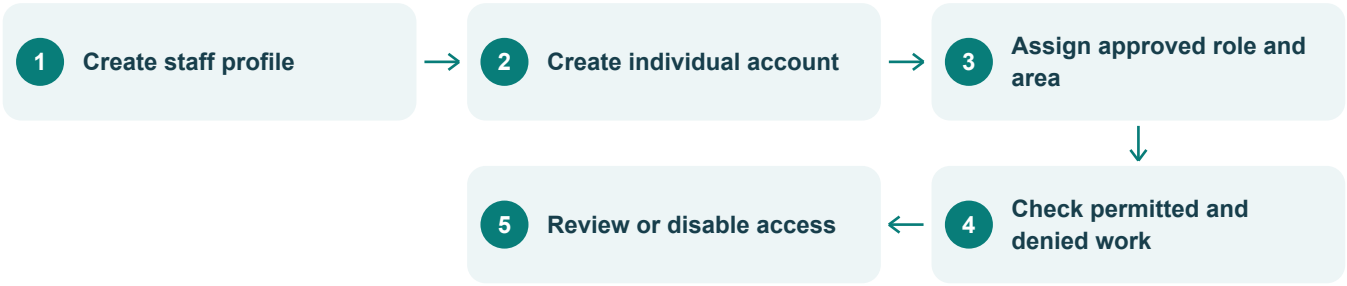
1. Confirm the clinic identity and company information through Your company or the available company screen. Keep display name, contact details and branding consistent.
2. Define facilities and geographic catchments. Check the association between facilities, clients and staff before importing or assigning work.
3. Confirm timezone, operating hours and public holidays. Review a sample appointment near a day boundary to catch timezone mistakes.
4. Create approved reference data, service categories and pricing before asking staff to quote.
5. Create staff profiles and individual accounts with the appropriate roles and employee links.
6. Verify a small complete workflow: enquiry, client, booking, assignment, visit completion and invoice review. Use approved practice records and avoid unintended external messages or financial transactions.

Clinic setup may show readiness steps or incomplete items. Treat each as a prompt to inspect the actual underlying configuration, not as a substitute for checking business results. Platform-level Customers or tenancy screens, if visible to a platform operator, are outside routine clinic administration; do not use them to manage a normal client/patient.

CHAPTER 02

People, roles and catchment access

The normal process



In this chapter

Create, change or disable an account

Access & roles, staff linkage and scope

Read the process from step 1 onward. Your role, the record status and the enabled features determine which actions are available. Check the saved result before handing work to the next person.

2.1 Create, change or disable an account

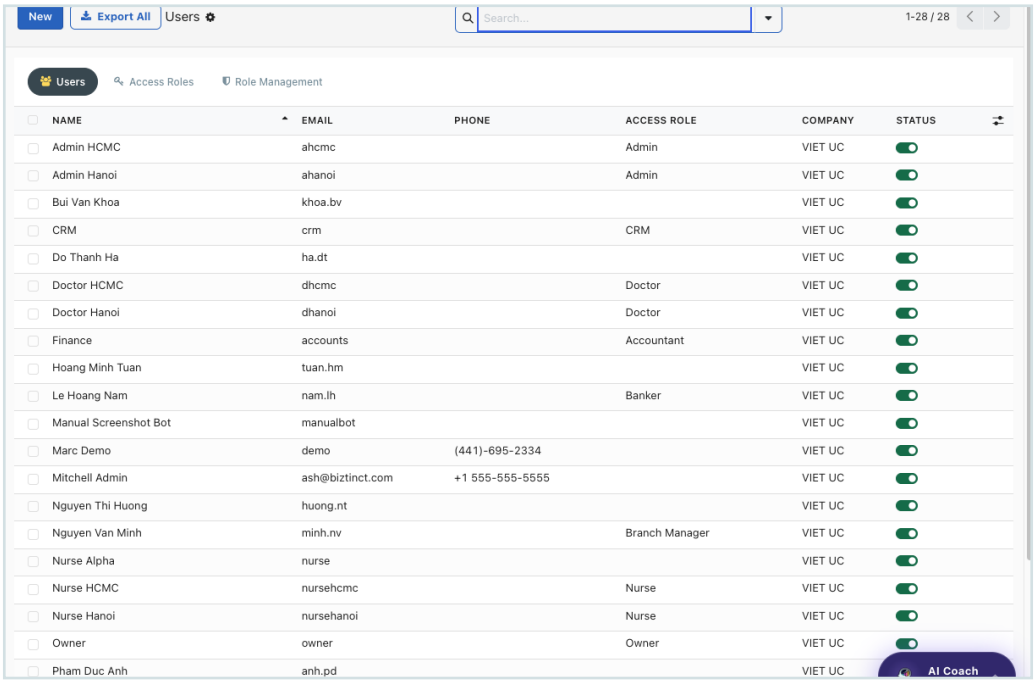


Figure 2. Users & Roles: locate the individual account before changing access.

Use Settings > People > Users & Roles. Search for the person first so one employee does not acquire duplicate accounts.

1. Create the named person with a clear individual login and the approved role. Follow the available account-activation or password process; do not put passwords in shared manuals.
2. Set company, facility and catchment as required. Link the employee record for nurse/doctor work so assignments match the login.
3. Save and ask the person to sign in. Check their starting screen and role-specific tasks.
4. Test one permitted action and one action they should not be allowed to perform. Check record visibility as well as menu visibility.
5. When someone leaves or changes duties, update the role or deactivate the account through the supported control. Reassign outstanding work and retain the history under the clinic's policy.

A login is the identity used to sign in. The employee record holds work-related details. The business role grants a defined set of functions. The catchment limits geography. All four must agree; fixing one does not automatically fix the others.

2.2 Access & roles, staff linkage and scope

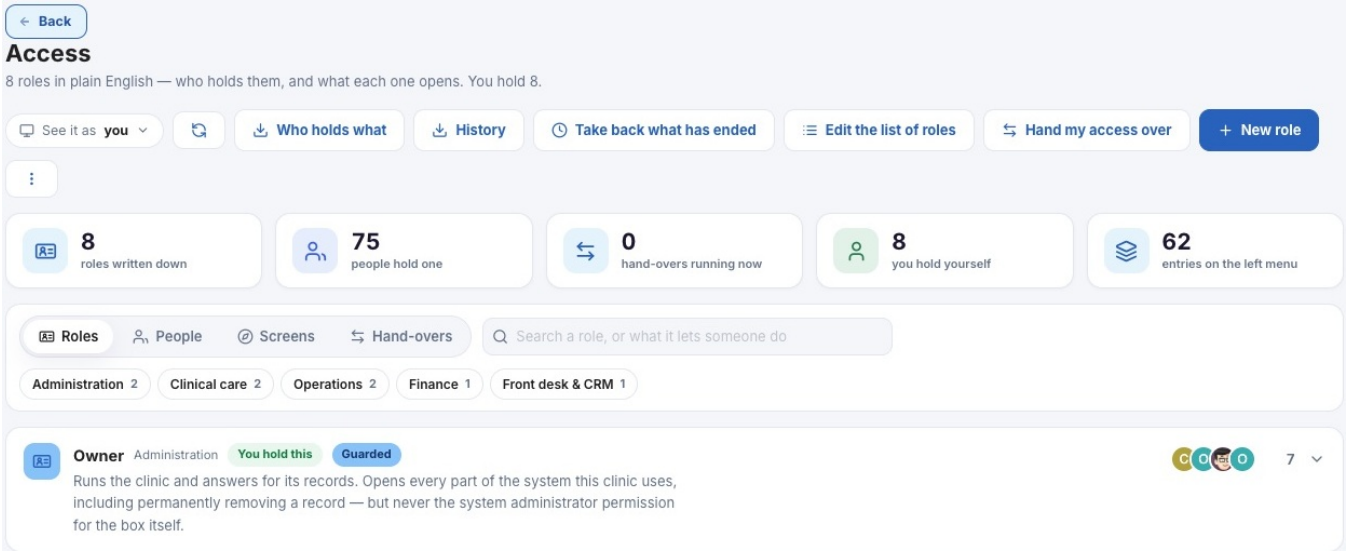


Figure 3. Access & roles: roles, people, screens and hand-overs.

Open Settings > People > Access & roles. The current access workspace explains roles, individual people, menu visibility and temporary cover. Use the relevant tab rather than changing a staff profile to solve a permission problem.

Tab	Use it to
Roles	Understand a business role and who holds it.
People	Inspect one person's roles and available screens.
Screens	Explain why a menu appears and which roles open it.
Hand-overs	Review access lent for temporary or ongoing cover.

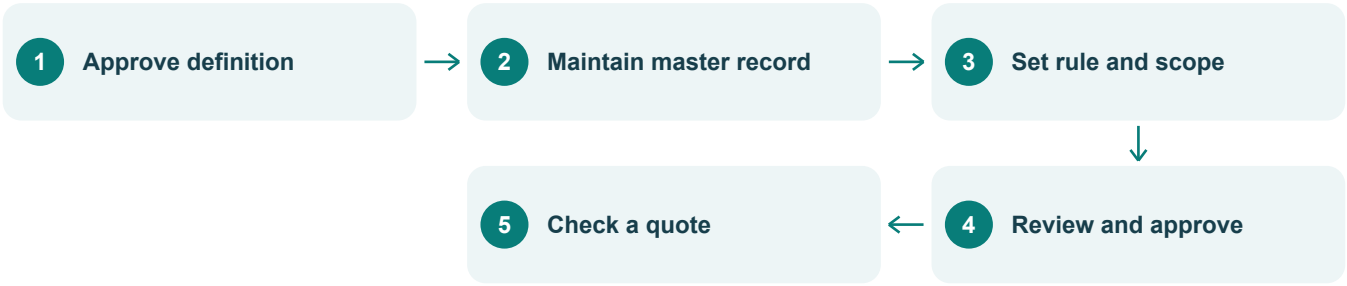
See it as previews another person's view without changing your login. Who holds what exports the current person-by-role picture. History records changes. Hand my access over starts the cover form. The detailed access walkthrough in the screen-by-screen guide explains these controls and the end-of-cover checks.

Check the login, business role, employee link and assigned catchments together. Operations Managers are limited to their assigned catchments; clinical visit work also depends on the relevant staff assignment. Healthcare Staff maintains the work profile, while this access workspace explains permissions. A visible menu alone does not grant access to every record.

CHAPTER 03

Master data, services and pricing

The normal process



In this chapter

Maintain approved dropdown values and equipment

Services, pricing engines and rules

Import a price list and maintain packages

Read the process from step 1 onward. Your role, the record status and the enabled features determine which actions are available. Check the saved result before handing work to the next person.

3.1 Maintain approved dropdown values and equipment

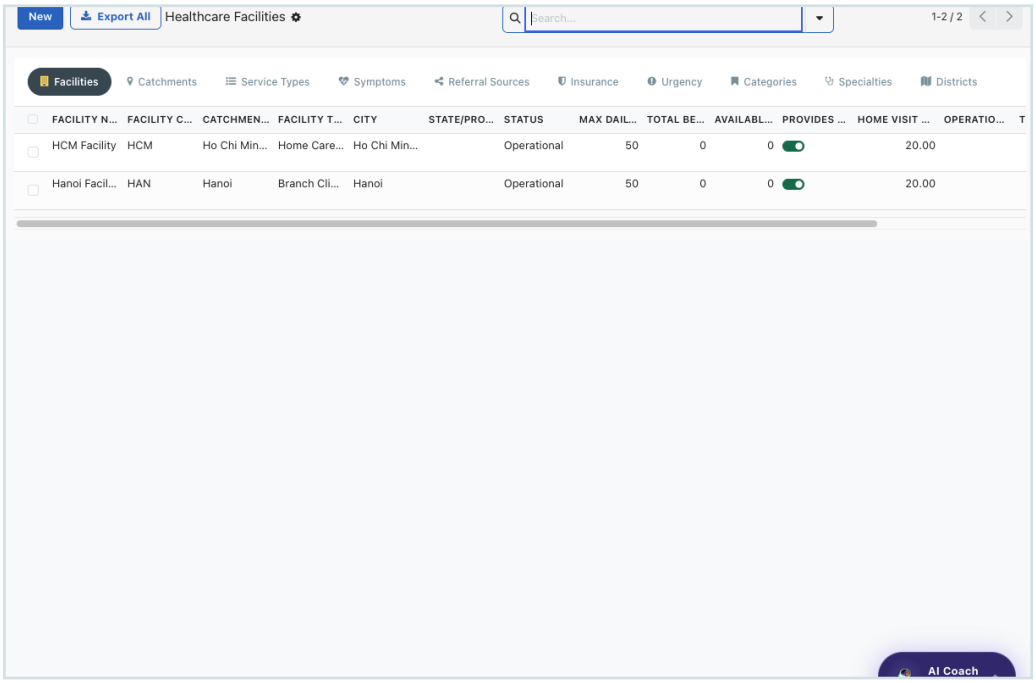


Figure 4. Master-data navigator: select the correct reference list.

Master data supplies values used across forms: facilities, catchments, service types, symptoms, referral sources, insurance, urgency, patient categories, specialties and districts where available. Different lists can be reached through the Facilities/master-data navigator and related tabs.

1. Search the existing list, including inactive entries if permitted, before adding a value.
2. Create a clear name and the required code or relationships. Use consistent spelling and avoid near-duplicate labels.
3. Save, then open the actual business form and confirm the new value is available in the correct dropdown.
4. For an obsolete value, prefer the supported archive/deactivate process when appropriate. Consider records already referring to it before deletion or renaming.

Equipment maintains portable items that can be attached to work. Record identity, availability and relevant service details, then verify selection on the booking. Public Holidays records dates used by scheduling and pricing; check the date, location/scope and applicability. A holiday entry can affect calculations, so verify both an ordinary day and the holiday after a change.

3.2 Services, pricing engines and rules

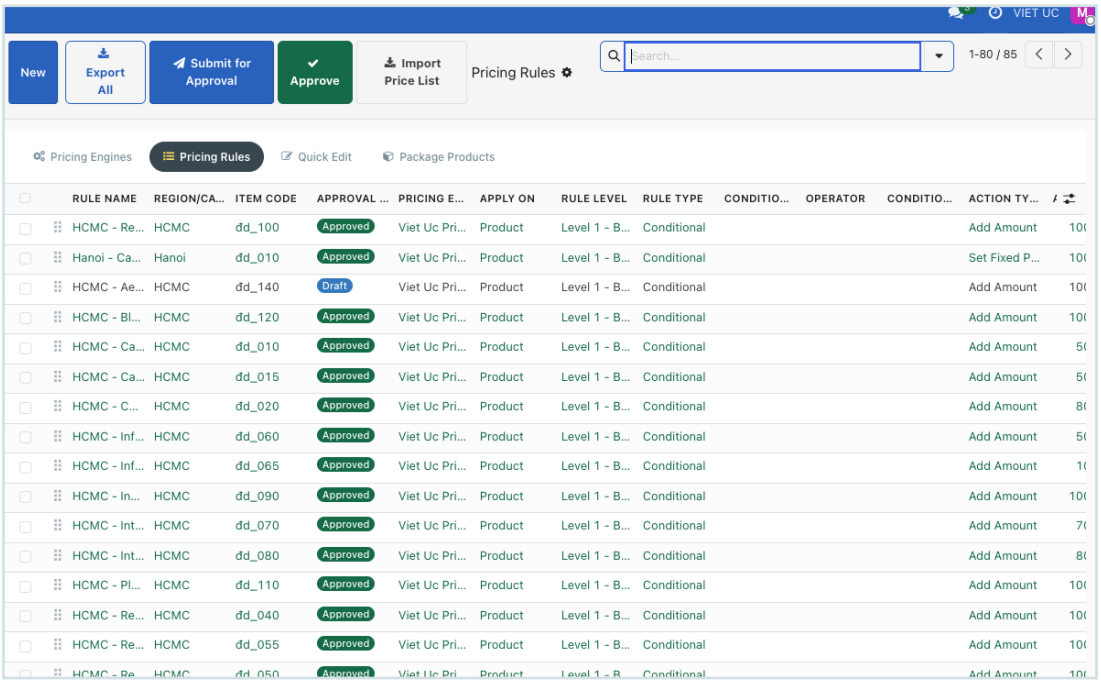


Figure 5. Pricing: distinguish rule editing, approval and price-list import.

Open Settings > Master data > Pricing. The area includes service/product information and pricing views such as engines, rules, quick edit and packages, depending on the selected tab.

A service/product identifies what is sold. A pricing engine groups calculation behaviour. A rule specifies when a price or adjustment applies. Rules can distinguish catchment, service, quantity or duration, distance, time, urgency and other configured conditions. Sequence and rule level can affect calculation order.

1. Confirm the approved tariff and effective scope with the finance owner.
2. Locate the existing service or rule by code and catchment. Check whether the change belongs to the base service price or an adjustment rule.
3. Update the relevant conditions and action, such as a fixed amount or surcharge. Check currency, unit, percentage and quantity interpretation.
4. Use Submit for Approval and Approve through the authorised process where those controls apply. A saved draft rule is not necessarily an approved active tariff.
5. Create or recalculate a suitable practice quote and inspect its breakdown. Check a case that should match the rule and one that should not.

Never judge success by the rule screen alone. The final service, travel, time and discount breakdown must agree with the intended calculation.

3.3 Import a price list and maintain packages

Import Price List opens the supported import wizard. Read its available template, matching options and import summary. A spreadsheet row must match the intended service or rule; identical names across different areas are not enough.

Before importing, retain an authorised copy of the current values and identify which codes and catchments will change. Check headings, amounts, units and duplicates in the proposed file. Import through the wizard, review errors and counts, then compare representative records against the approved file. Quick Edit changes records more rapidly but still needs the same review.

Check examples across ordinary weekdays, weekends, holidays, evening/night time, travel boundaries, extra hours, discounts and added services when those rules apply. Confirm no unintended duplicate rules are contributing twice. Old invoices and quotations may retain historical values; do not assume a tariff update should rewrite historical transactions.

For packages, define the correct included services, entitlement and validity. Check sale, payment, use on an eligible visit and remaining entitlement. Keep product definitions separate from a client's purchased package and usage history.

CHAPTER 04

Forms and clinical configuration

The normal process



In this chapter

Form Studio and required fields

Clinical form templates and medical catalogues

Menu layout and clinic settings

Read the process from step 1 onward. Your role, the record status and the enabled features determine which actions are available. Check the saved result before handing work to the next person.

4.1 Form Studio and required fields

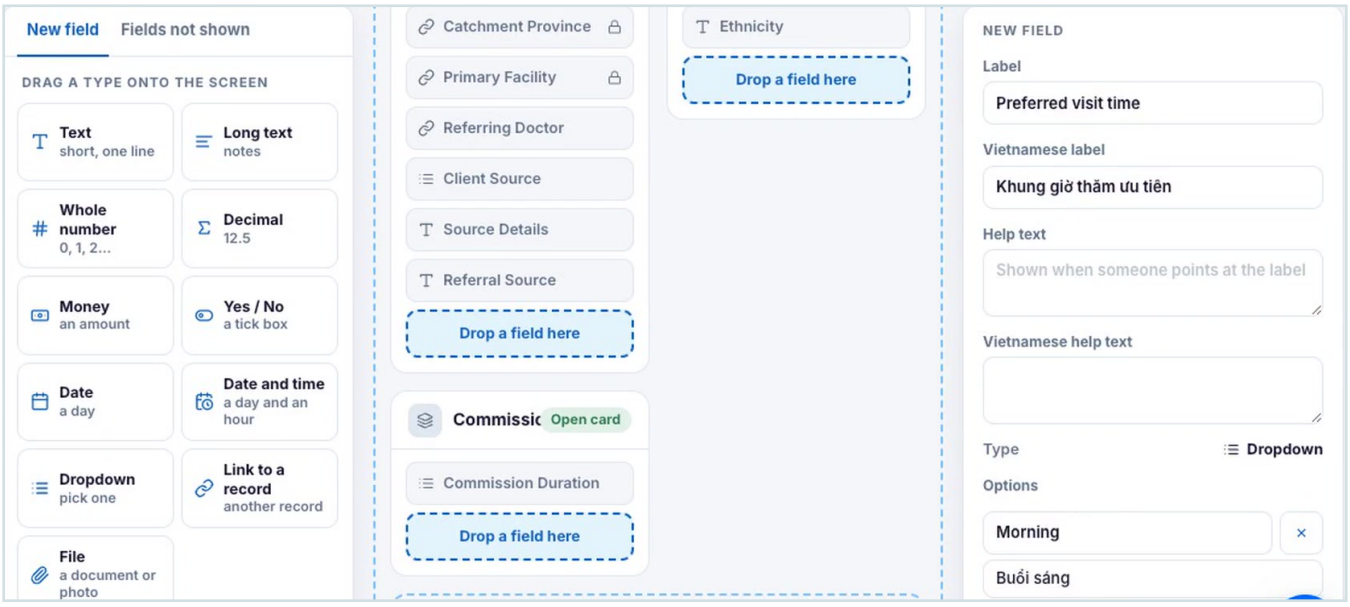


Figure 6. Form Studio: define a field and review its type, label and options.

Form Studio customises supported business forms and lists, including Booking, Client, Contact and Incident. Field Requirements controls when information is mandatory. These are different tools: one changes structure or placement; the other controls completion requirements.

1. Identify the exact form/model and intended audience. Write down the business purpose, field type and label before editing.
2. Open Form Studio, select the supported form and add or place the field using its editor. Choose an appropriate text, number, selection, date or boolean type.
3. Review visibility, placement and available publishing controls. Preview the form before applying the layout.
4. If the field must be mandatory, configure the correct role and state conditions in Field Requirements. Avoid making it required before staff can reasonably know the answer.
5. Check a new record, an existing record and the relevant mobile or alternate screen. Confirm data is saved and the field does not block unrelated roles.

Changing a dropdown label or removing a field can affect reporting and existing data. Keep the meaning stable and plan the correction with the business owner. Do not use a new custom field as a substitute for an existing standard field with the same purpose.

4.2 Clinical form templates and medical catalogues

Form Templates defines clinical assessments. A template contains questions, answer types, required flags and optional scoring bands. Publish makes an approved version available; Retire removes it from future use through the supported lifecycle; New Version supports controlled change. Preserve the meaning of completed assessments.

1. Have the clinical owner agree the wording and scoring.
2. Create the draft questions and options, including units and required conditions.
3. Preview a complete example and a missing-answer example. Check the calculated band and any extracted observation mappings.
4. Publish the approved version and verify it appears for the intended visit workflow.

Observation Types defines structured measurement types, units and terminology links. Thresholds are clinical configuration and must be approved by the clinical lead. Medication Catalog defines medicines referenced by orders; maintaining a catalogue does not prescribe a treatment. Monitoring Devices links a device and client, with active, suspended and retired states. Check the identity, expected source and latest readings before treating incoming values as reliable.

Use Suspend or Retire through the supported device action when appropriate. Investigate data-source errors separately from an alert about a client's condition.

4.3 Menu layout and clinic settings

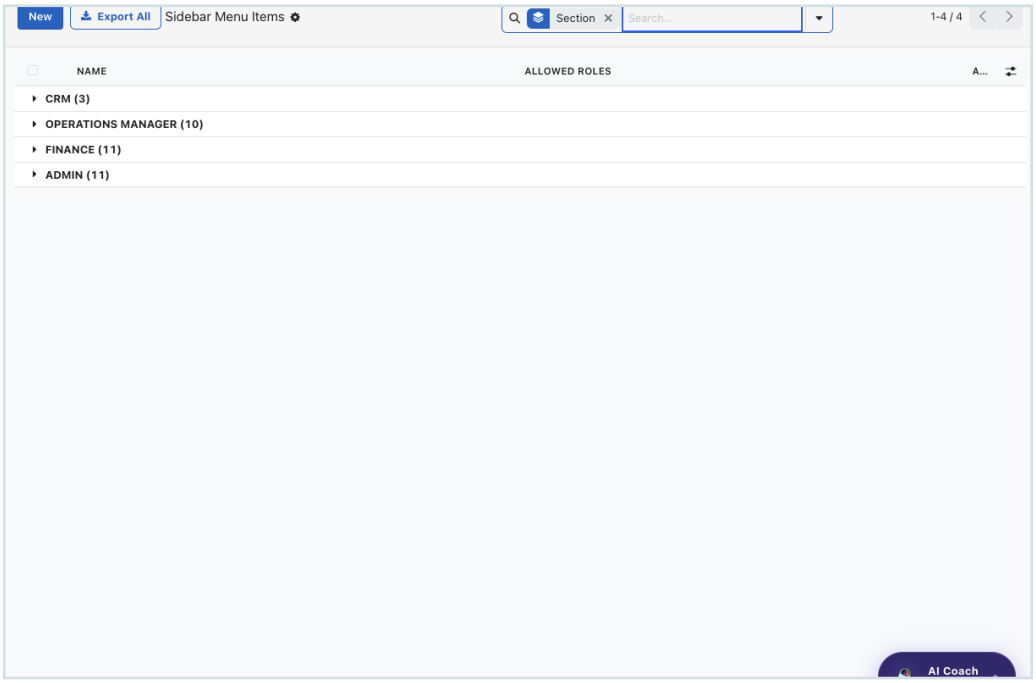


Figure 7. Menu configuration: check the destination action and grouping.

Settings > Menu controls navigation entries, sections and grouping. A heading may group several screens; an item points to a particular action. Renaming or moving an item changes navigation, not the underlying model permissions.

Before changing a menu, identify its existing action and intended roles. Keep the structure understandable: area, tab, then screen. Check that a renamed item still opens the correct screen and that child screens remain reachable. Review both English and Vietnamese labels where maintained. Do not expose an administrator screen to ordinary users as a workaround for an access error.

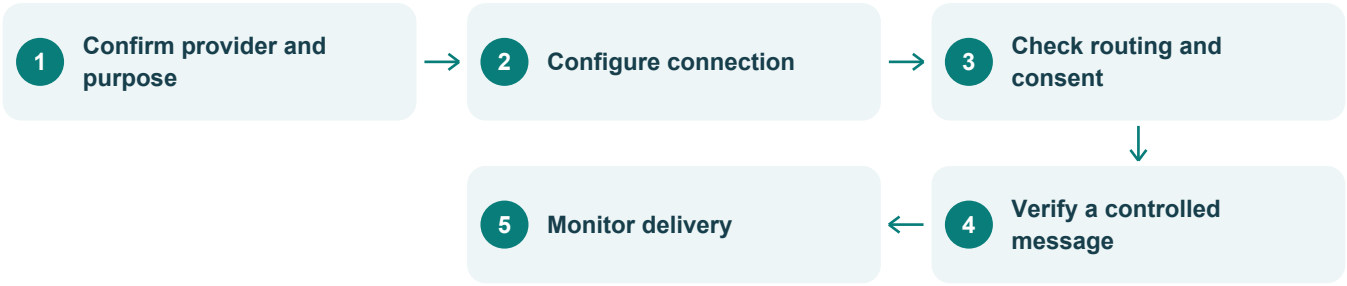
Settings contains company-wide options such as maps, billing and integrations, depending on installed modules. Change only the option belonging to the intended company and workflow. After a change, check a representative client or booking rather than relying on a successful Save message.

About and Activity history help identify the workspace and review changes. Record the reason for a configuration change so a later administrator can understand it.

CHAPTER 05

Connections, communications and AI

The normal process



In this chapter

Channels, phone and website enquiries

Reply templates, reminders and family access

AI assistance and what remains a human decision

Read the process from step 1 onward. Your role, the record status and the enabled features determine which actions are available. Check the saved result before handing work to the next person.

5.1 Channels, phone and website enquiries

Settings > Connections contains Channels (setup), Channel Go-Live, Website Connector, Phone system and Extensions where enabled. These screens configure services; seeing a menu does not mean a provider is connected.

For each channel, identify the clinic account, permitted purpose, responsible owner and routing destination. Follow the provider-specific connection workflow, store credentials only in the designated fields, and check connection status. Use Channel Go-Live to review the available readiness checks before actual use.

Phone system and Extensions map the provider account and staff extensions. Verify incoming-call routing, call history and permitted recording access with the phone owner. Website Connector configures how website enquiries enter the clinic; confirm the intended site, destination and attribution fields. Google Ads application configures the provider application separately from the connected advertising account shown to CRM.

After setup, use an authorised example message or enquiry and trace it through Unrouted Contacts, Channel messages, Channel audit and the resulting contact. Confirm errors and retries do not create duplicates. Live-channel operation requires the actual provider accounts and approved connection; a saved configuration alone is not evidence of live delivery.

5.2 Reply templates, reminders and family access

Reply Templates provides reusable wording; Watchlist phrases supports configured recognition in channel work. Have the responsible team approve wording and language. Check placeholders and a real preview so the wrong client name or date is not sent.

Visit Messaging maintains outgoing confirmation/reminder records and their outcomes. Its enabled setting and dry-run mode determine whether messages are actually sent. A dry-run record is a simulation, not delivery. The implemented routing can use configured Zalo ZNS, email or staff-call follow-up according to availability and outcome; inspect the record rather than assuming every channel was used.

Family links, portal access and self-booking invitations are tied to a client or visit. Verify recipient identity, consent, scope and validity. Use the supported revoke or token-rotation action when access should end or a link is exposed. Do not paste private links into broad group conversations.

Review failed delivery, fallback and duplicate-prevention behaviour with the operational owner. Changing a template does not correct a message already sent; record the correction and communicate it through the approved process.

5.3 AI assistance and what remains a human decision

AI functions have different purposes. Care Command may suggest a reply. Voice Notes may produce a transcript. Coding Review may suggest a medical code. Coaching may suggest a summary or action. The learning coach retrieves published instructions. Do not assume enabling one feature approves every type of data processing.

Use the existing approved provider configuration and processing-destination controls. Identify what information each feature sends, who may use it and what consent or approval is required. Verify the feature's enabled state and failure behaviour before making it available to staff.

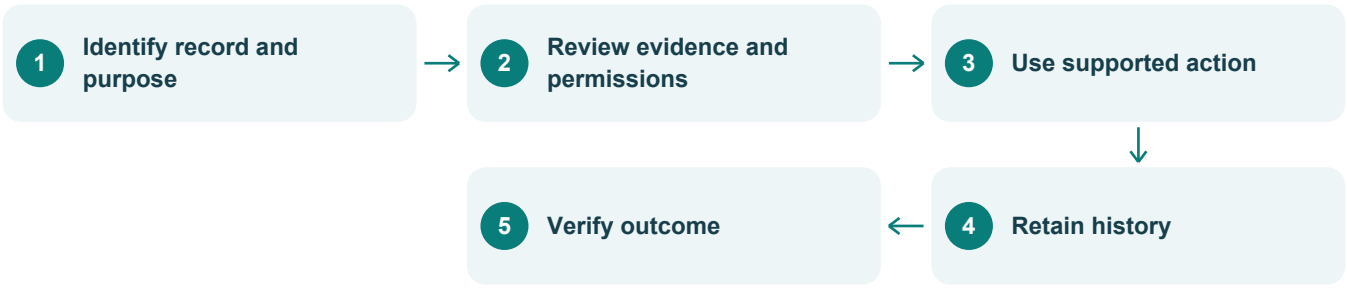
The user must review a reply before sending, a transcript before saving as the clinical note, a proposed code before approval and a coaching recommendation before adopting it. The learning coach cannot perform business actions or replace clinical judgement.

When an AI feature is unavailable, use the supported manual workflow. Do not copy client records to an unrelated consumer AI service to bypass a configuration or consent restriction. Review logs only with the appropriate permissions and avoid exporting sensitive payloads unnecessarily.

CHAPTER 06

Records, compliance and controlled corrections

The normal process



In this chapter

Audit Log, consent checks and data lifecycle

Compliance Register and evidence

Rights requests, calendars and retention holds

Processing destinations and authority integrations

Terminology, EMR export, readiness and EVV

Read the process from step 1 onward. Your role, the record status and the enabled features determine which actions are available. Check the saved result before handing work to the next person.

6.1 Audit Log, consent checks and data lifecycle

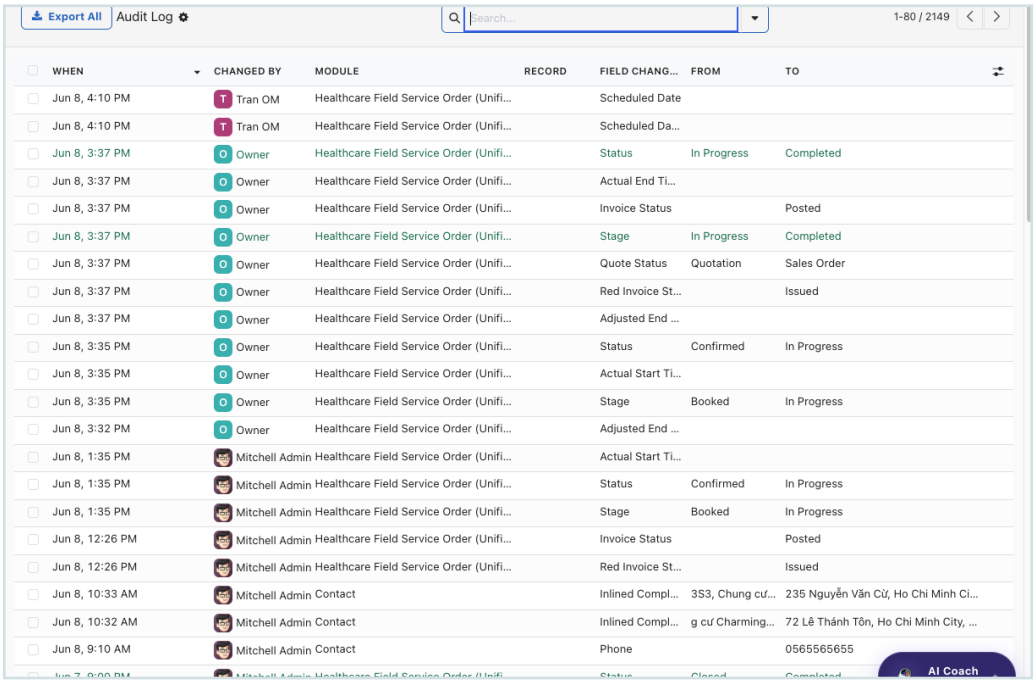


Figure 8. Audit history: narrow by record and date, then inspect the event.

Settings > Records > Audit Log helps trace who changed a record and when. Use the model, reference, person and date filters to narrow the search. Review the event and linked record together; a log entry must be interpreted in context.

Consent Check Log records available permission checks. Use it when investigating why a sharing or processing action was allowed or blocked. Do not edit the consent simply to make a past failed action appear valid.

Data Lifecycle records supported archive, deletion and restoration activity. Archiving removes a record from ordinary active lists; it is not necessarily permanent deletion. A deletion action can have stricter permissions and requirements. Read the impact, reason and any retention hold before proceeding. Use restore only where the application offers a supported recovery route.

For important corrections, retain the original reference, reason, responsible person and resulting record. A final clinical record should use its permitted addendum process. Review the actual corrected output after the action, not just the absence of an error message.

6.2 Compliance Register and evidence

Compliance Register organises requirements and their supporting evidence. Technical verification, operational/manual evidence and authority approval are separate dimensions. A working software feature is not, by itself, proof that an external authority has approved it.

Evidence Artifacts records supporting documents and their scope. Enter a meaningful description, the relevant requirement, date and responsible owner. Approve after review should only be used after the authorised reviewer has checked the actual evidence. Preserve reviewed records and use the available correction process when information changes.

1. Open the requirement and read the expected outcome.
2. Check which evidence relates to the current company, environment and period.
3. Add the relevant proof through the permitted evidence action.
4. Review the separate statuses and unresolved items.
5. Export the register only for an authorised audience and check what the export includes.

The register is a work and evidence tool. It does not automatically establish legal compliance or national-system acceptance. Assign an owner to unresolved requirements and track the next action.

6.3 Rights requests, calendars and retention holds

Rights Requests supports requests concerning personal information. Record the requester, subject, request type, received date, responsible owner and verified identity information through the permitted fields. Handle identity evidence through the clinic's approved process.

Record acknowledgement requires proof of the actual acknowledgement. Record extension notice captures an actual notice when an extension is authorised. Complete request should follow the completed outcome and its communication evidence. A computed deadline helps organise work; the responsible privacy lead must confirm the applicable requirement and any exception.

Rights Calendar maintains the relevant working/non-working-day information used by the workflow. Retention Holds prevents selected data from being removed while a legitimate hold applies. Record the scope, reason and responsible owner. Release hold only when the authorised decision permits it.

Before deleting or exporting information in response to a request, check identity, scope, clinical-record obligations and active holds with the responsible specialist. Keep a record of what was provided or corrected. Do not treat a request as permission to expose unrelated people's information.

6.4 Processing destinations and authority integrations

Processing Destinations identifies recipients or services that may receive information for an approved purpose, such as AI processing or support. Review purpose, scope and supporting approval before using Approve destination. Do not grant a broad destination merely to make one integration work.

Authority Integrations tracks the readiness of external capabilities such as certified signing or national-health connections. Record the actual provider, contract/onboarding status and evidence. A configured adapter or locally generated file is not evidence of a successful production authority connection.

Where the screen distinguishes pending, configured or approved states, preserve that distinction in operational reports. Do not enter an approval state without the required proof. Review changes with the responsible clinical, privacy or integration owner.

Secrets, access tokens and provider credentials belong only in the intended secured configuration controls. Never include them in screenshots, training examples, exported user manuals or broad chat messages.

6.5 Terminology, EMR export, readiness and EVV

Terminology contains Medical Codes, Coding Systems and Import Codes. Maintain the approved coding system and version. Check code format, descriptions and duplicates when importing. Changing a code's meaning can affect existing clinical interpretation.

EMR (VN) > Readiness shows available checks for the Vietnam export profile. Resolve missing identity, facility or clinical information through its proper source record. Export prepares the supported file/bundle and records a submission-log entry. In the current adapter, file export is distinct from live national transport; a local export is not proof that a national service received or accepted it. Record actual acknowledgement only when supported by real evidence.

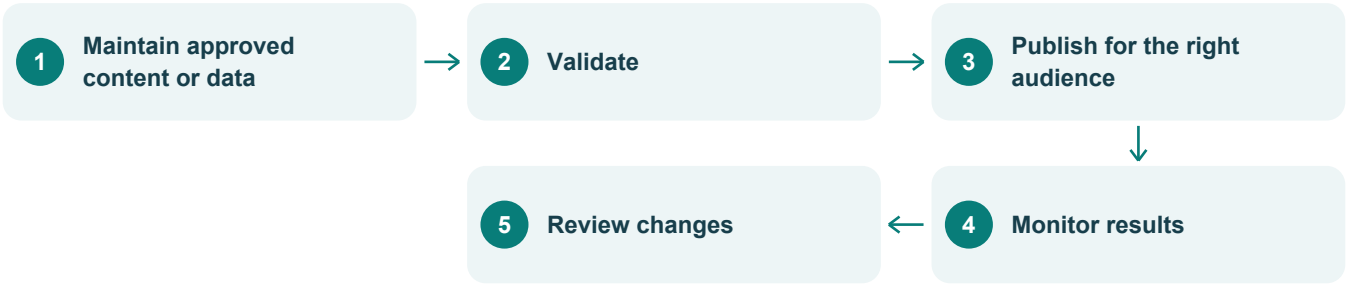
EVV Events provides visit verification history. Inspect the linked visit, event time, location and other available evidence. The event log supports traceability; it is not a screen for silently rewriting check-in history.

When investigating a missing export or verification event, preserve the reference and error, check the source data and consult the integration owner. Do not invent identity, signatures, timestamps or acknowledgement details to make a readiness check pass.

CHAPTER 07

Reporting and learning administration

The normal process



In this chapter

Analytics sources, datasets and dashboards

Maintain training and interpret learner progress

Routine review and safe troubleshooting

Read the process from step 1 onward. Your role, the record status and the enabled features determine which actions are available. Check the saved result before handing work to the next person.

7.1 Analytics sources, datasets and dashboards

Analytics > Data groups Sources, Datasets, Refresh jobs, Pipelines, Semantic model and Glossary. These are reporting configuration tools and should be managed by the reporting administrator rather than ordinary dashboard readers.

A source identifies where data comes from. A dataset defines the data made available for reporting. A pipeline prepares it. The semantic model defines business measures and relationships. The glossary explains terms. Refresh jobs bring the data up to date. A dashboard presents the selected result.

1. Confirm the approved source and access scope. Use Test Connection where available without exposing credentials.
2. Define or review the dataset and transformation. Validate before applying or publishing.
3. Check row counts, dates, measures and role restrictions using known records.
4. Publish only the intended dataset/dashboard and set recipients and schedules carefully.
5. Monitor refresh outcomes; investigate errors before users rely on stale figures.

A refreshed dataset can still contain an incorrect business definition. Compare key totals with source invoices, bookings or payments. Do not grant direct source access to solve a dashboard-permission problem.

7.2 Maintain training and interpret learner progress

Learn contains two related experiences. Task guides and videos uses the curated task library, role paths and guide-grounded coach. Training uses lessons and stations with its own learning records. Training admin includes Lessons, Stations, Learner progress, Learning events and Training wording where available.

Maintain a named owner for each learning topic. When a workflow changes, check the written steps, screenshots, captions and video chapters together. Retire or update outdated instructions rather than adding contradictory notes. Review the learner-facing wording in both languages.

Use learner progress to identify people who may need help. Reviewed, watched or quiz-completed flags do not prove clinical competence or grant application permissions. Combine them with supervised practice and the clinic's role assessment.

The curated task/video catalogue is maintained through the application's content release process, rather than by editing an arbitrary lesson record. Request a coordinated update for its guides, video files and role mappings. Keep videos within their intended access and provide administrator manuals only to the appropriate audience.

7.3 Routine review and safe troubleshooting

Daily: check unresolved delivery failures, failed refresh jobs, important access reports, pending cash and operational exceptions with the responsible teams. Weekly or at the clinic's agreed interval: review leavers and changed roles, expiring consent, upcoming holidays, outdated training and unresolved compliance evidence.

When a problem is reported, record the user, role, company, catchment, screen, reference, time and exact message. Reproduce with the least privilege needed. Separate missing data, wrong filters, wrong assignment, wrong permissions, stale data and a genuine software defect.

After a fix, check the original case and a neighbouring case that should remain unchanged. For an access change, confirm both permitted and prohibited work. For pricing, check a matching and non-matching quote. For a form, check existing records as well as new ones. For integration, verify the external outcome as well as the local status.

Keep regular users focused on their tasks. Share the user manual and relevant guides; keep this administrator manual in the designated restricted location. Escalate code/deployment issues to the technical operator with the evidence needed to investigate.

Screen-by-screen guide

Use the earlier chapters to understand the business process. Use this guide to look up each menu, its purpose and its main controls. Paths use Area > tab > submenu. Some tabs group screens rather than opening a separate record. Your role, company, catchment and enabled features determine which entries you see.

This directory covers administrative and restricted operational screens. Everyday care, bookings, billing and the learner journey are described in the separate user manual. Access & roles has a detailed walkthrough at the end of this guide.

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Each entry explains what the screen is for and how to recognise the next action. Read the current record and status before using a button; the controls available can change with your role or the record state.

Compliance

Define the requirement > maintain evidence > review readiness > track outstanding action.

Compliance > Terminology

Terminology groups medical codes, the coding systems they belong to and controlled code import. Keep vocabulary maintenance separate from coding an individual patient note.

Compliance > Terminology > Medical Codes

Medical Codes is the terminology catalogue used when selecting clinical codes. Search by code or description and check the coding system and active status. Use a defined code rather than creating a duplicate label. Catalogue maintenance belongs to the responsible administrator or terminology owner.

Compliance > Terminology > Coding Systems

Coding Systems defines the vocabularies to which medical codes belong. Check the system name and identifiers before importing or interpreting codes. The same-looking code can mean something different in another system. Maintain this catalogue with the terminology owner so reports and exported records use the intended vocabulary.

Compliance > Terminology > Import Codes

Import Codes loads an approved terminology file into the selected coding system. Confirm file format, system and expected scope, then use the import form and review its result. Check sample codes afterwards. Importing clinical terminology changes shared lookup data, so obtain the clinic's approval and avoid duplicate or unverified files.

Compliance > EMR (VN)

EMR (VN) groups export history and readiness checks. Fix missing source information before preparing the next export.

Compliance > EMR (VN) > Export

Export records prepared EMR/FHIR export submissions and their outcomes. Inspect client or record references, destination, state and error details before retrying. Follow the configured export workflow and check the resulting file or response. Creating an export file is separate from delivery to, and acceptance by, an external authority.

Compliance > EMR (VN) > Readiness

Readiness checks whether required information is present for the configured Vietnamese EMR export. Read missing items and resolve them in their source records, then recheck. A readiness result concerns the implemented checks; it does not by itself certify legal compliance or external-system acceptance.

Compliance > Visit Messaging

Visit Messaging is the outbound-message queue for visit-related communication. Check recipient, related visit, channel, scheduled time, state and error before following up. Verify consent and recipient details in the source record. Do not repeatedly resend a queued item without checking whether the provider has already accepted it.

Compliance > EVV Events

EVV Events records electronic visit-verification events such as check-in and check-out. Inspect event time, visit, worker and available location or verification information. Compare exceptions with the assignment and actual visit record. It is an evidence log; a missing or unusual event needs investigation rather than an invented replacement timestamp.

Compliance > Compliance Register

Compliance Register separates technical implementation, manual clinic action and external authority dependencies for each requirement. Open an item to read applicability, owner, outstanding action and evidence. Export register creates a review copy. Do not treat a technical status alone as confirmation that the clinic's operational or external obligations are complete.

Compliance > Evidence Artifacts

Evidence Artifacts stores supporting files and summaries for compliance requirements. Check the linked requirement, kind, environment, release, limitations and expiry. Approve after review records an explicit review decision. The file should support the stated claim; uploading a document alone does not prove the requirement has been satisfied.

Compliance > Rights Requests

Rights Requests tracks an individual's information-rights request. Verify identity and scope, assign an owner and check acknowledgement and completion due dates. Record acknowledgement and Record extension notice retain supporting proof. Complete request records the outcome and evidence. Use the clinic's authorised process for the decision; this register tracks it.

Compliance > Retention Holds

Retention Holds records instructions to retain a record class or client's records. Check scope, retain-until date, reason and active status. Release hold requires the appropriate release reason and authority. A hold affects retention decisions; it is not a request to erase the record or an instruction to bypass the clinic's policy.

Compliance > Rights Calendar

Rights Calendar maintains non-working dates used by the rights-request deadline process. Check date and name before adding or correcting an entry. This calendar is separate from staff leave and the public-holiday pricing calendar. After an authorised correction, review affected request deadlines rather than assuming every date has been recalculated.

Compliance > Processing Destinations

Processing Destinations records where information may be processed or sent. Check endpoint, country, purpose, legal basis, processor terms and validity. Approve destination records the authorised review. A registered destination is not automatically an active connection; check the corresponding integration and intended data flow separately.

Compliance > Authority Integrations

Authority Integrations records external integration readiness: provider, status, specifications, onboarding evidence and pending action. Keep credential location as a reference, not a place to paste secrets into general notes. Review supporting evidence before changing status. This register describes readiness; it does not itself transmit data to an authority.

Analytics - data administration

Choose a business question > check the data > analyse > explain and share the result.

Analytics > Data

Data groups the reporting foundation: sources, datasets, refresh jobs, pipelines, model and glossary. Administrators maintain these; report readers use the resulting dashboards.

Analytics > Data > Sources

Sources defines where analytical data comes from. Check source type, model or view, connection and sync status. Test Connection checks connectivity; Sync Now requests a refresh; Pipeline Editor opens transformation steps where supported. Keep connection secrets restricted and review the intended data scope before applying a source change.

Analytics > Data > Datasets

Datasets defines a reporting model built from sources. Review nodes, relationships, fields, row grain and company scope. Visual Modeler helps inspect the structure; Scan Fields refreshes field discovery. Publish or Re-publish makes the model available to reports; Unpublish removes that availability. Check totals and joins before publishing a changed dataset.

Analytics > Data > Refresh jobs

Refresh jobs controls scheduled dataset refresh. Check dataset, interval, state, last refresh, duration and next run. Refresh Now requests an immediate update when permitted. After running, inspect success and freshness rather than assuming the dashboard changed. A failed refresh can leave an otherwise working report showing older data.

Analytics > Data > Pipelines

Pipelines is the visual editor for preparing source data. Inspect each transformation in order, including filters, calculated values and joins. Validate before applying a change, then compare sample rows and totals. A pipeline affects downstream datasets and reports; coordinate changes with the reporting owner.

Choose the source at the top. + Add Step inserts a transformation. Validate & Apply checks and applies the pipeline; the cleaned-data preview shows the result. Review the preview before relying on downstream reports.

Analytics > Data > Semantic model

Semantic model explains how reporting sources relate and how fields are used. Review the root, relationships, cardinality, dimensions and measures. Use it to keep business definitions consistent rather than building a different meaning into every chart. Validate duplicate counting and company scope before publishing or using a changed model.

Open in Explore opens the selected model for analysis. Rescan Fields refreshes discovered fields. Inspect business name, type, role, default aggregation, folder and visibility in the field panel. Re-publish updates the published model after review.

Analytics > Data > Glossary

Glossary is the shared dictionary for report terms. Search the term and read its definition, formula note and owner. Use the same meaning when presenting a dashboard or comparing clinics. If a definition is unclear, ask its owner before changing it; wording alone does not automatically change a dataset calculation.

Learn - training administration

Find a topic > read and watch > practise > check understanding.

Learn > Training admin

Training admin groups learning content, journey structure, progress, activity records and clinic wording. It is separate from the learner's Training screen.

Learn > Training admin > Lessons

Lessons maintains the content within a training station. Review goal, duration, ordered steps, tips and understanding checks. Each check needs the intended answers and useful feedback. Preview the learner experience after changes. Keep screen names and instructions aligned with the application and maintain both languages.

Learn > Training admin > Stations

Stations organises the training journey. Set the title, section, sequence, duration and prerequisite or previous station where supported. The outline explains what, why, when and prerequisites; lessons supply the steps. Check active and required settings and preview the journey so learners are not sent to a missing or unsuitable next station.

Learn > Training admin > Learner progress

Learner progress shows each person's station state, position, attempts and completion time. Use it to identify who may need help or where to resume. Compare the actual learning activity before interpreting repeated attempts. Do not treat course completion alone as proof of competence or permission to perform restricted work.

Learn > Training admin > Learning events

Learning events is the timeline of learning interactions. Filter by learner, station, event kind, screen, language or time to investigate progress or usability questions. It supports the progress record but is not a clinical record. Limit access to those responsible for learning support and use the event context before drawing conclusions.

Learn > Training admin > Training wording

Training wording stores clinic-specific wording overrides. Check the key, value, note and company before editing. Change only the intended phrase and preview affected lessons in each language. An override changes the explanation shown to learners; it does not change the underlying business workflow or access permissions.

Settings

Identify the setup need > update the correct definition > review scope > verify the affected work.

Settings > Overview

Overview is the administrator's starting dashboard. Use its shortcuts to the relevant setup area and inspect any attention items. Confirm the current company before editing shared data. This overview points to work; each linked configuration screen has its own save, approval or validation process.

Settings > People

People separates login accounts, access explanations and assignable healthcare staff profiles. Check all three when onboarding or changing a person's duties.

Settings > People > Users & Roles

Users & Roles maintains named login accounts. Search for the person first, then check active status, login, company and assigned access. Use the supported creation or deactivation process and link the correct staff record. Account changes and staff scheduling are separate: when disabling someone, review future assignments and delegated access too.

Settings > People > Access & roles

Access & roles opens the Access & delegation workspace. Its current CareJiyo tabs are Roles, People, Screens and Hand-overs. Use Roles to understand abilities, People to inspect an individual, Screens to explain menu visibility, and Hand-overs to review temporary cover. See it as previews another person's access without signing in as them. The detailed access walkthrough follows this directory.

Settings > People > Healthcare Staff

Healthcare Staff maintains the work profile used for assignment: employee, role, skills, facility, service area and availability details. Confirm its link to the person's login before assigning visits. Changing a staff record does not automatically grant every menu permission; check business roles and catchment access separately.

Settings > Master data

Master data contains reusable definitions used throughout the clinic: facilities, prices, equipment, calendars, clinical catalogues and form controls. Review downstream records before changing a shared definition.

Settings > Master data > Facilities

Facilities defines the clinic locations used by staffing and care workflows. Check name, address, contact and associated geographic information. Search before adding another facility with a similar name. Review affected staff and client links after a correction so reports and appointments continue to point to the intended location.

Settings > Master data > Pricing

Pricing opens the service/product catalogue and its pricing workflow. Check service, unit, active status and linked rules before changing a price. Use the relevant effective dates, area, staff/service conditions and approval controls. Validate a representative quote after an authorised change. A catalogue label, a price rule and an already posted invoice are different records.

Settings > Master data > Equipment

Equipment is the portable-equipment register used by the clinic. Review identification, type, location, availability and related operational details shown on the form. Keep an existing item's identity stable when updating its status. Check the affected visit or assignment when equipment availability changes.

Settings > Master data > Public Holidays

Public Holidays maintains calendar exceptions used in scheduling and related rules. Confirm the date range, description and applicable calendar. Review holiday-sensitive quotes and staff planning after a change. This is not the Rights Calendar used for information-rights request deadlines.

Settings > Master data > Observation Types

Observation Types defines what a measurement means: name, value type, units and coding details. Maintain the catalogue with clinical oversight. Check existing observations and forms that use a type before changing it. A unit or type change can alter interpretation; do not repurpose an existing type for a different measurement.

Settings > Master data > Medication Catalog

Medication Catalog defines medicines that prescribers can select. Check the medicine identity, units and available safety/reference information. Avoid duplicate spellings for the same medicine and follow the clinic's approval process. Updating the catalogue does not itself authorise a prescription or record an administration.

Settings > Master data > Form Templates

Form Templates defines assessment questions and scoring. Review Questions, Scoring Bands, Description and Preview; maintain English and Vietnamese labels. Publish makes an approved version available, Retire stops its use, and New Version supports controlled changes. Existing assessments keep their captured template context; do not rewrite historical answers by changing a template.

Settings > Master data > Monitoring Devices

Monitoring Devices links a device to the correct client and external identifier. Check device type, vendor, state, last reading and receipts. Suspend pauses an unsuitable device; Reactivate restores it; Retire ends its use. Recheck the client mapping before accepting readings after reassignment. Device registration alone does not prove readings are arriving.

Settings > Master data > Form Studio

Form Studio is the visual editor for supported business forms. Choose the form, add or place fields, set type, English/Vietnamese label and options, then preview and apply through the available controls. The field palette includes text, numbers, dates, money, yes/no, dropdowns, links and files. Review existing records and role visibility after changing a layout.

The landing library lists custom fields across All screens, Contact, Booking, Incident and Client. Open Contact/Booking/Incident/Client in Studio opens the corresponding editor. An empty library means no custom fields are listed; it does not mean those business forms are missing.

Settings > Master data > Field Requirements

Field Requirements controls when information must be supplied. Select the supported record type and review the role/state conditions for each requirement. Require information only at a stage when the user can reasonably provide it. Check a new record, an existing record and the relevant mobile workflow after a change; a misplaced requirement can block completion.

The first screen groups the configurable sections into Sales & CRM, Bookings & Assignments, Finance and Admin. Choose the section and record type before editing requirements.

Settings > Connections

Connections groups messaging, website, advertising and telephone setup. Keep provider credentials and activation work with authorised administrators.

Settings > Connections > Channels (setup)

Channels (setup) maintains provider connections and routing. Check channel, company, connected resource and connection status. Configure credentials only through the intended restricted fields, then validate inbound routing and authorised outbound delivery. Keep setup separate from the day-to-day conversation queue; a saved connection may still need provider approval or configuration.

Settings > Connections > Reply Templates

Reply Templates maintains reusable message wording. Review the title, content, language and intended situation before saving. Check any placeholders against the actual client or conversation when using the template. A template saves typing; it does not replace consent, recipient checks or the staff member's responsibility to review the message.

Settings > Connections > Channel Go-Live

Channel Go-Live is the guided readiness workspace for channel activation. Follow the displayed steps to review connection, routing and readiness evidence. Distinguish a local configuration check from an actual provider exchange. Resolve failed checks before relying on the channel for clients, and verify results in Channel messages and Channel audit.

Provider cards cover Meta, Zalo, Google and Microsoft. Start or Continue opens that provider's guided application-registration steps. The Calls section links to Channel Center because phone setup follows a different provider process. Read readiness and pending steps rather than assuming all providers are enabled.

Settings > Connections > Website Connector

Website Connector defines how authorised website forms send enquiries to the clinic. Check website/origin, connection settings, field and catchment mapping, and the displayed validation results. Confirm that a submission creates or links the intended contact and touchpoint. Keep integration secrets out of public page content and avoid mapping the caller as the patient by mistake.

Settings > Connections > Google Ads application

Google Ads application is the restricted platform configuration for the advertising integration. Its credentials and application-level settings are separate from the clinic's advertising account and reports. Platform administrators maintain it. Clinic staff should use CRM > Web & Ads > Google Ads and ask the administrator to resolve application-connection issues.

Settings > Connections > Phone system

Phone system maintains the connection to the telephony provider. Check provider settings, connection state and synchronisation options in the form. Coordinate changes with the phone administrator and confirm calls appear with correct direction, matching and timing. Configuring a provider does not automatically assign every extension to a staff member.

Settings > Connections > Extensions

Extensions maps phone extensions to the appropriate user or staff member. Verify extension number, owner and active status before saving. Review reassignment when somebody changes duties or leaves. Correct extension ownership helps route callbacks and explain call activity; it does not change the person's clinical or finance role.

Settings > Records

Records groups business audit history, archival history and consent-check evidence. Choose the log matching the event you are investigating.

Settings > Records > Audit Log

Audit Log records supported changes to business records. Filter by time, model, record or user, then inspect the recorded action and details. Use it to investigate what changed and follow the link to the source when authorised. Audit coverage depends on what the application records; the absence of a row is not proof that nothing happened.

Settings > Records > Data Lifecycle

Data Lifecycle is the archive/lifecycle history. Review the affected record, action, actor and time to understand retention or archival activity. Check the source and applicable holds before taking further lifecycle action. Archiving, recoverable deletion and permanent removal have different consequences; use the clinic's approved procedure for each.

Settings > Records > Consent Check Log

Consent Check Log records whether a consent check passed or failed for an attempted activity. Inspect client reference, type, scope, result, consent, user and source. Use it to explain why a communication or other action was permitted or blocked. Correct consent in its own record; this log is evidence of the check, not the consent itself.

Settings > Menu

Menu maintains the navigation entries: label, section, parent, order and destination action. Check both the top-level tab and its child entries before changing placement. Menu configuration determines where a screen appears; access rules determine who may use it. Preview the affected roles and confirm links still open the intended screen after a change.

Settings > Settings

Settings opens the clinic's configuration workspace. Use the named categories and read each setting's explanation before changing it. Record the business purpose and verify the affected workflow after saving. Not every option belongs to everyday clinic users; leave technical or provider-specific settings to the responsible administrator.

Its tiles are General Settings, Company Profile, Access & roles, Pricing Engines, Field Requirements and Form Studio. General Settings opens system-wide options; Pricing Engines opens pricing-rule configuration. The other tiles lead to the corresponding dedicated screens described in this guide.

Settings > About Viet Uc Care

About Viet Uc Care identifies the current clinic workspace and available system information. Use it when reporting a support issue so the correct organisation and environment are identified. The menu name can reflect the clinic's branding; CareJiyo is the product name. This information screen is separate from editing company setup.

The release timeline explains listed updates. Support access can be switched on or off by an authorised person; enabling it allows the platform support process to create time-limited support access with a recorded reason. Review the displayed support-session history. This switch changes access and should follow the clinic's approval process.

Settings > Your company

Your company is the clinic organisation profile. Review identity, contact, address and the available company information before editing. Use the provided save controls and check the displayed result. This is where company information is maintained; it does not replace facilities, staff, prices or integration setup.

The form includes company name, address, phone, email, website, tax/registration numbers, country and logo. Save company details saves the profile; Refresh reloads it. The document preview shows the entered identity. Currency and platform settings are managed separately by the platform administrator.

Settings > Clinic setup

Clinic setup is the guided starting checklist for preparing a clinic workspace. Open each setup area, review what is complete and follow the link to maintain the underlying record. Return to check the result. A checklist helps coordinate preparation; it does not prove every clinical, billing or external integration workflow has been verified.

The six steps are Company, Facilities, People, Services, Connections and Review. Select a step or use Previous step/Next step. Open takes you to the related configuration; Setup notes and Save progress keep the checklist notes/progress. Refresh updates the displayed readiness.

Settings > Activity history

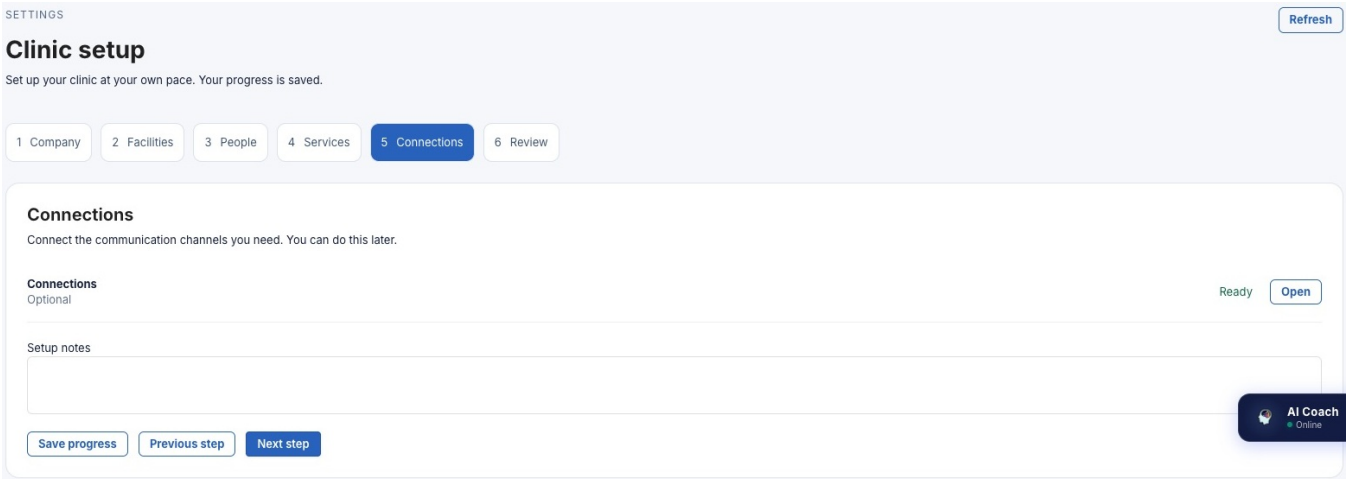
Activity history combines several permitted history sources into one timeline. Search summaries, choose a source and use Refresh, Previous or Next to review events. Source availability is shown separately. Access still follows each original record's rules; detailed clinical content and full request payloads are not displayed in this overview.

Settings > Customers

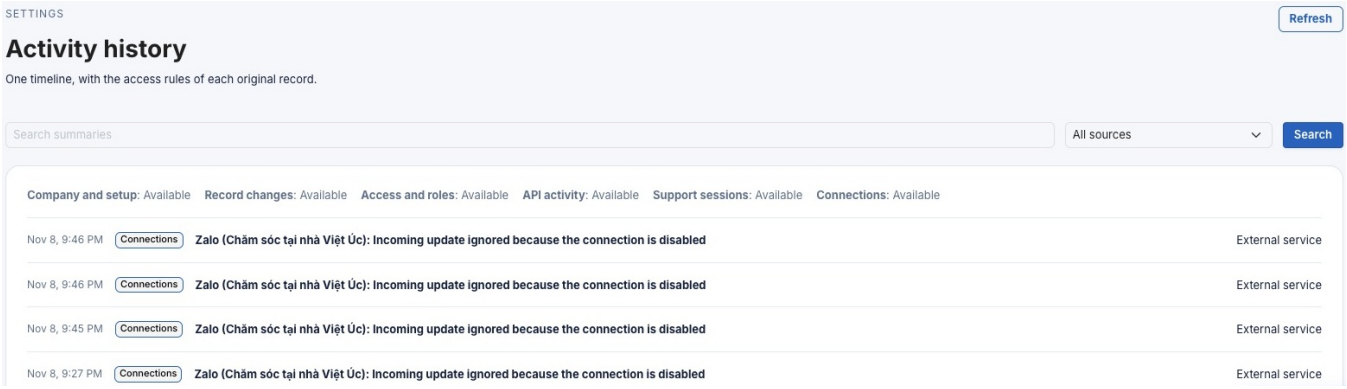
Customers is the restricted workspace for managing customer/tenant organisations. It is intended for platform-level administration, not for registering patients. Clinic care recipients belong in Operations > Clients. If this menu is visible to you, confirm your platform responsibility before changing an organisation or its service settings.

Refresh reloads customer health. Alerts shows attention items. Rollout manages releases; What each customer has reviews installed capabilities; Plans and invoices manages service plans; In step with master compares versions. New customer starts provisioning. Customer rows link to the workspace and messaging controls. These are platform operations and can affect other organisations.

Clinic setup and activity history

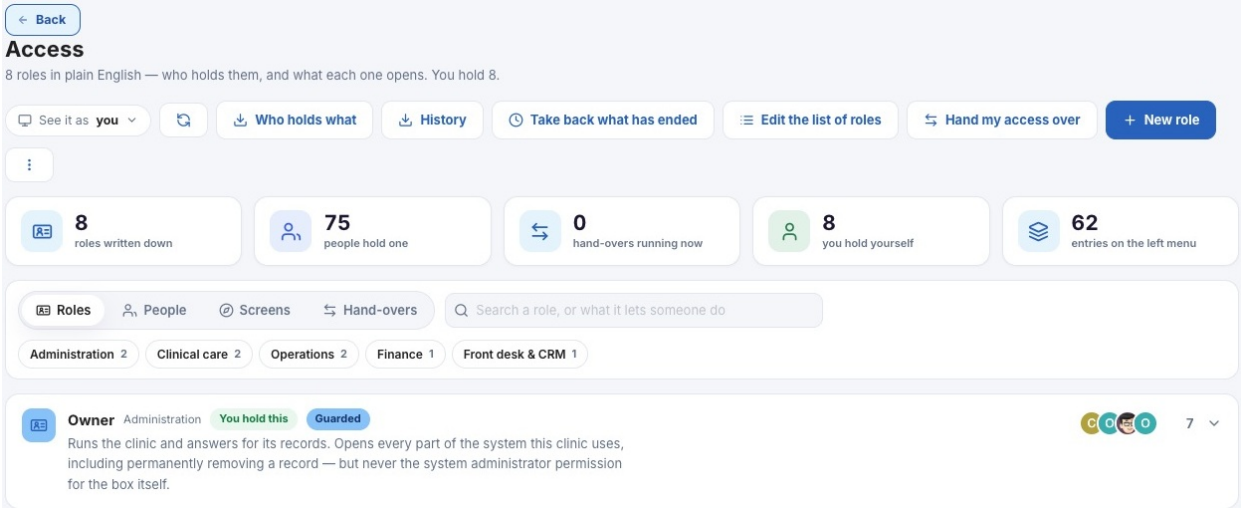


Clinic setup keeps the six preparation steps together. Open goes to the underlying configuration. Save progress records the checklist work; it does not itself validate every workflow. Previous step and Next step move through the checklist.



Activity history combines the permitted sources. Search summaries and the source filter narrow the timeline; Refresh reloads it. An event reports what happened and who or which service did it. Use the source record when more detail is needed.

Access: roles and people



Open Settings > People > Access & roles. The page title is Access and the workspace is also called Access & delegation. Start by deciding whether you need to understand a role, inspect a person, explain a menu, or arrange cover.

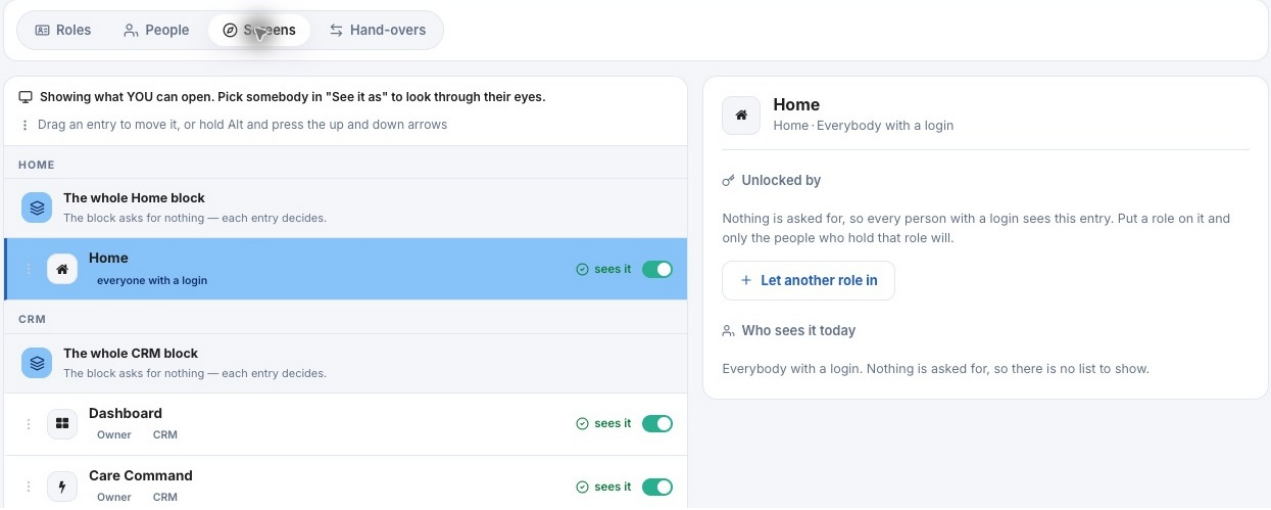
Control	What it means
Roles	Search a job role, expand it and read the abilities and people who hold it.
People	Search a person, select them and review their roles and the screens available to them.
See it as	Preview another person's access explanations. Your own login and authority do not change. Return to You afterwards.
Who holds what	Download the person-by-role report for review. Store it with the appropriate access restrictions.
Refresh / Read it again	Reload the displayed information after a change.

In Roles, read the whole role description before giving it to somebody. In People, select the intended individual and check their existing access first. Use the available grant or removal control, review the confirmation and check the resulting list. Only make a change authorised by the clinic.

New role and Edit the list of roles maintain the business-role catalogue. The role editor groups abilities and shows the resulting menu. A role name alone does not tell you what it grants. You cannot use this workspace to hand out the whole-system administrator permission, and you cannot grant abilities you do not hold.

The login, business role, staff record and catchment each serve a different purpose. After a change, confirm the person's allowed task and an out-of-scope record. Operations Managers remain limited to their assigned catchments.

Access: understand who can see a screen



Choose Screens to inspect the menu. The left side lists areas and entries; selecting one shows why it is visible and who can see it. Expand the relevant area or entry to find its children. Use See it as when investigating a particular person's view.

1. Select the person whose access you are investigating, or leave the view as You.

2. Find the relevant area, tab and submenu. Check whether it is switched off or has a role gate.

3. Read Unlocked by and Who sees it today. A role on an area can affect its entries; inspect inherited access as well as the entry itself.

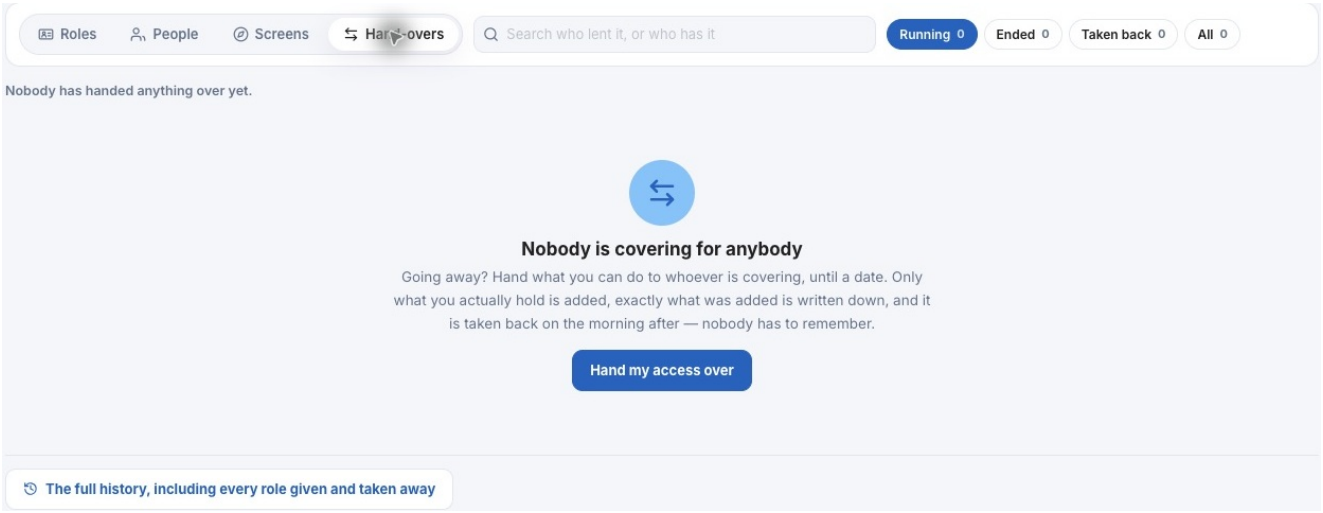
4. If a change is authorised, use the relevant role or menu-visibility control and review the resulting explanation. Check the person's actual task afterwards.

5. Return See it as to You. Refresh if the displayed information is older than a recent change.

Control or label	Meaning
On/off switch	Whether the menu entry is placed on the menu. Hiding an entry is not the same as revoking all access to its records.
Let another role in	Changes which role can open the selected entry. Review the scope before confirming.
Drag / Alt + arrow keys	Changes menu order where supported; it does not change the business meaning of a record.
Through an area / inherited access	The permission comes from a parent area rather than only this entry.
Everybody with a login	The entry has no role gate of its own in the displayed explanation.

A visible menu and permission to read or update an individual record are different checks. Company, catchment, assignment and record rules still apply. Do not broaden a role simply to bypass a legitimate record restriction.

Access: arrange and end temporary cover



Hand-overs records access lent to another person for cover. The normal flow is: choose the person covering, select the permitted roles, set the dates, explain why, confirm, then review and end the cover.

1. Choose Hand my access over. An authorised manager may also arrange cover for a person who is away through the available person selector.

2. Find the person who will cover. Check their identity; you cannot hand access to yourself.

3. Select only the roles needed for the cover. The lender must actually hold the complete permissions being lent.

4. Choose For a while for temporary cover. Enter From and Until and check the order of the dates. Choose For good only for an authorised ongoing arrangement; it has no automatic end date.

5. Enter Why. Read the summary and use Hand it over only when the arrangement has been approved. Cancel closes the dialog without confirming it.

6. Return to Hand-overs and check the people, roles, dates and state. Running, Ended, Taken back and All filter the history.

7. Use Take it back now when cover ends early. Review the result and the recipient's remaining access.

Temporary access is eligible for automatic removal after the Until date through the scheduled check. Take back what has ended runs the due-expiry check when an authorised administrator needs it. Confirm that the hand-over ended; do not assume that a date alone proves the scheduled job ran.

The system tracks what a hand-over added and takes that addition back. The recipient may still have the same capability through their own role or another valid arrangement. Ending one hand-over is not the same as disabling the person's login.

Access: review history and investigate a problem

Use Who holds what for the current person-by-role picture. Use History or The full history, including every role given and taken away for changes and hand-overs. These answer different questions: who has access now, and how it changed.

Question	Where to look
What does this role permit?	Roles: expand its explanation and abilities.
What can this person see?	People, then Screens with See it as.
Why is one menu absent?	Screens: visibility, role gate and parent-area access.
Why can they see a menu but not a record?	Account/company, role, staff link, catchment, assignment and the record's own scope.
Who is covering and until when?	Hand-overs: Running and the arrangement details.
Why did access remain after cover ended?	The person's own roles and other active hand-overs, then the recorded removal result.
What changed?	History and, where relevant, Settings > Activity history or Audit Log.

For an onboarding review, confirm one named login, the correct employee link, the approved role and the assigned catchments. For a role change, remove unneeded access as well as adding the newly approved access. For a departure, review future assignments and current hand-overs before completing the account-deactivation procedure.

Keep downloaded access reports restricted. They can expose names and organisational responsibilities. Review them with the clinic owner or access administrator and record the agreed action through the normal process.

The access workspace explains and manages permissions. It does not create clinical authorisation, professional qualifications or approval to work outside the clinic's assigned scope.